جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : عزيزه محمد مصطفي زيدان

المحافظة : القاهره **الحي / المدينة :** حلوان **العنوان :** مساكن صلاح سالم

رقم البطاقة : 28304160102661 **رقم التليفون :** 01147115367 **تاريخ تسجيل الإستمارة :** 23-10-2026

العمر(تاريخ الميلاد) : 16-04-1983 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**

التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج كيماوي Anthracyclines base [14151] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: عزيزه محمد مصطفي زيدان
National ID: 28304160102661

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Anthracycline-based chemotherapy**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية

عزيزه
محمد مصطفى زيدان
ش ادهم-مساكن صلاح سالم-ب ١-م ١
حلوان - القاهره

١٩٨٢/٠٧/١٦
٢٨٣ ٠٤ ١٦ ٠١ ٠٢٦ ٦١
JR6917579

DMS
تاكول وكابو
~~شروط~~
DMS الورقيه



٢٨٣٠٤١٦٠١٠٢٦٦١ ٢٠٢٢/١٢
حاصله على دبلوم معهدخدمه اجتماعيه

انثى مسلمة متزوجة
ناصر رضوان عويس محمد
١ البطاقة سارية حتى ٢٠٢٩/١٢/٠٥

مبـــادرة رئيس الجمهوريـة
لدعـم صحة المرأة المصرية

06/24/2026

MDT FORM

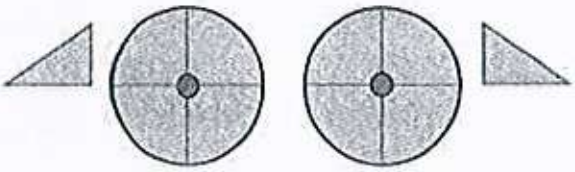
Name:	عزيزه محمد مصطفى Zedan	**MRN:**	11053
Age:	43 Y	**ID:**	28304160102661
Menopausal Sta	pre	**Phone:**	01147115367
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:	free	**Reason Of MDT:**	new

Clinical Data:

BX: IDC G 2 + DCIS

ER: 0/8 **PR:** 0/8 **HER2:** -ve by s/s **KI67:** 80%

R L



TNM

Sonomammography:
sonomamo:
Opinion:
. Left breast pathologically proven neoplastic mass for correlation to histopathological results (BIRADS 6 • Mild fibrocystic changes of right breast (BIRADS 2)

Cesm
IMPRESSION:
Left breast: Large inner breast pathologically enhancing malignant lesion associated with skin thickening and focal edematous changes as described above (BIRADS 6).
Right breast: Few probably benign lesions, for ultrasound follow up after 6 months (BIRADS 3).

Mets Workup:
ct:
OPINION:
Left breast mass. ? Left axillary enlarged LNs. > Riaht breast retroareolar benign looking tiny lesion ? Right axillary reactionary looking LNs > No lung metastasis.
? Mid line pelvic cyst is seen indined on right side with thin wall with no septations likely right adnexal cyst. ? Mild pelvic fluid is seen at the douglas pouch. > No hepatic or bony suspicious lesions.

History:
clinically: LT breast UIQ large breast mass with central edema ?? most propably for possibility of neoadj ttt

Recommendations:

MDT Outcome & Plan

For:
1.Bone scan
2.Echo
3.Portacath insertion
4.Start NACT "Keynote 522 protocol"...Taxol/Carbo x 12 followed by AC x4, with concomitant Keytruda q 3 weeks....followed by Lt SM & SLNB.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د / شهاب لاشين	د / اشرف عيسوي				

24/6/2026

Name: عزيزه محمد مصطفى زيدان **MR. :** 11053	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

Prem left breast cancer ct4b no mo idc 2 er0/8 pr0/8 her22+ sish not amplified ki 80% triple-negative brca1 2 pending on exam left breast edema redness peau d orange left breast mass m 15x15 for w1 taxol 140 carbopl auc 2 c1 keytruda 200 if approved (wt 68 ht 170 sa 1.78) ef 68% cesm rt birads 3 benign lesions

For w2 taxzol 140 carboplatibn auc 2 and c1 keytruda 200 when available w3 ph

On exam left breast mass 12 x12 cm left axillary ln 1x2 for w4 taxol 140 carbopl auc 2 ,w5 ph pt did not receive keytruda yet bec not available ,there is approval

Pharmacy Visit Wt: 68, Ht: 170 ,BSA: 1.78, AdBwt: 64.1 For W5 Taxol 140 + Carboplatin AUC 2 Labs 10/8 .. accepted

for w5 taxol carboplatin + c1 keytruda, w6 taxol carboplatin pharmacy with cbc, w7 pharmacy

Pharmacy Visit W6 TC + C1 Keytruda was on 7/9 Today For W7 Taxol 140 + Carboplatin AUC 2 LAbs CBC7/9 .. ANC 1.3 FOr GCSF Functions 31/8 .. accepted

The patient has completed 7 weeks of the taxol 140 + carboplatin AUC 2 regimen.

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

GUSTAVE ROUSSY
INTERNATIONAL HOSPITAL

Patient Name	: الأستاذه / عزيزه محمد مصطفى زيدان	Request No.	: 13006031
Patient No.	: 28304160102661	Request Date	: 21/04/2026
Path No.	: 1118/26	Verification Date	: 12/05/2026
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 43 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast UIQ mass.
Nature of Specimen: Tru-cut biopsy, collected on 21/4/2026, at 11:30 am, Cold ischemia time: 20 seconds.

Gross Examination:
- Fragmented grey white tissue cores, ranging in size between 0.2 cm & 0.8 cm long, totally submitted.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
* Ductal carcinoma in situ: Intermediate grade, (Solid pattern).
* Perineural invasion: Absent.
* Angioinvasion: Absent.

Diagnosis:
Left UIQ breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 2.
- Intermediate grade ductal carcinoma in situ (Solid pattern).
- For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Maha Guimei.

Verified by :
Dr Maha Guimie

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY
INTERNATIONAL CANCER CAMPUS

Patient Name	: الأستاذه / عزيزه محمد مصطفى زيدان	Request No.	: 13006031
Patient No.	: 28304160102661	Request Date	: 21/04/2026
Path No.	: 1118/26	Verification Date	: 12/05/2026
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 43 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yous

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast UIQ mass diagnosed Invasive duct carcinoma, Grade 2, with intermediate grade ductal carcinoma in situ (Solid pattern).
Nature of Specimen: Tru-cut biopsy, collected on 21/4/2026, at 11:30 am, Cold ischemia time: 20 seconds.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Results:
* IPT (All controls showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified.
(Average Her2 copy No.: (3.6), Her2/CEP17 ratio: (1.89)).
- Ki67%: 80%.

Diagnosis:
Left UIQ breast mass, Tru-cut biopsy:
IPT:
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified, Group V.
(Average Her2 copy No.: (3.6), Her2/CEP17 ratio: (1.89).
- Ki67%: 80%.

Verified by :
Dr Maha Guimie

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient's Name:	**Aziza Mohammed Mostafa Ziad**
Date:	**2026-06-02**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Clinical History:

- **New case of left breast cancer for work up.**

Findings:

- Left breast mass is seen measuring 7x7.2cm.
- Left axillary enlarged LNs largest 16x15mm.
- Right breast retroareolar benign looking tiny lesion is seen measuring 1cm.
- Right axillary reactionary looking LNs.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen on left side.
- Right minimal pleural thickening and effusion.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

OPINION:

- Left breast mass.
- Left axillary enlarged LNs.
- Right breast retroareolar benign looking tiny lesion.
- Right axillary reactionary looking LNs.
- No lung metastasis.

Patient's Name:	**Aziza Mohammed Mostafa Ziad**
Date:	**2026-06-02**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

- Right minimal pleural thickening and effusion.

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION: -

Findings:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Uterus is of average size with no focal lesions.
- Mid line pelvic cyst is seen inclined on right side measuring 10x9cm with thin wall with no septations likely adnexal cyst.
- Mild pelvic fluid is seen at the douglas pouch.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses.

Opinion:

- Mid line pelvic cyst is seen inclined on right side with thin wall with no septations likely right adnexal cyst.
- Mild pelvic fluid is seen at the douglas pouch.
- No hepatic or bony suspicious lesions.

Dr. Mai Shaaban MD

Patient ID: 96590	Patient Name English: Azizah Mohamed Mostafa
Age: ٤٣ سنة, ٢ شهر	2026/06/24 10:13:51 AM :Report Date

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: 43 years old female patient with pathologically proven IDC grade II + DCIS (-ve ER, -ve PR, +ve HER2 and Ki67= 80 %)

TECHNIQUE: CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background uptake: mild background enhancement.

ACR: C (heterogeneously dense breast parenchyma).

Left breast:

- **Low energy (Non contrast) images show**: Large inner breast round indistinct hyperdense lesion associated with skin thickening.
- **Recombined (Post contrast) images show**: corresponding inner quadrants mainly non-uniform rim enhancing lesion that measures about 8.3x9.8x8.7 cm being about 3 cm from the nipple.
- **Complementary US**:
  - Upper inner quadrants 8-11 o'clock palpable heterogeneous hypoechoic lesion with indistinct margin, abutting the overlying thickened skin and associated with focal edematous changes.

Right breast:

- **Low energy (Non contrast) images**: multiple obscured oval shaped isodense lesions. No suspicious microcalcifications.
- **Recombined (Post contrast) images** show UOQ, UIQ and retro-areolar few circumscribed homogenously enhancing lesions.
- **By Targeted ultrasound:** corresponding circumscribed oval parallel homogenous hypoechoic lesions are seen follows:
  - At the UOQ (11 o'clock), measuring 1.1x0.5 cm and 0.6x0.3 cm.
  - At the UIQ (1 o'clock), measuring 0.8x0.4 cm
  - At the retro-areolar region, measuring 1.2x0.7 cm.
- Normal skin / nipple / areolar complex.

IMPRESSION:

Left breast: Large inner breast pathologically enhancing malignant lesion associated with skin thickening and focal edematous changes as described above **(BIRADS 6).**

Right breast: Few probably benign lesions, for ultrasound follow up after 6 months **(BIRADS 3).**

Best regards,

Dr. Sherihan Fakhry, MD

Dr. Amira Shaheen, MD

Name : Aziza Mohammed Mustafa Zidan
Date : 13-06-2026 07:22
Branch : Dokki
Medical No. : 392210
Ref.Physician Prof : Goustav Roussey Egypt

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

History

43 year-old female patient , presented with history of left breast cancer and referred for metastatic workup .

Technique

- **Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mci of the tracer.**

Findings

- **No evidence of focal active or cold osseous lesions that would account for bone secondaries could be seen.**
- **Degenerative changes are noticed at different levels of the spine and around big joints.**

IMPRESSION

- **No osseous metastases detected on scintigraphic basis.**

Much obliged
Dr. Alaa Nagy, MD
Nuclear medicine

NILE DIAGNOSTIC CENTER

أ.د. ناجى عبد الوهاب
أستاذ الأشعة - جامعة القاهرة

الاســـــم :.	عزيزه محمد مصطفى	تاريخ الفحص :.	16/6/2026
الســـــن :.	44	كود المريض :.	698079
جهة التحويل :.		رقم العمليـــة :.	11260614143
الطبيب المعالج أ.د.:.			

Thank you for referring this case,

Digitalized Low Dose Soft Tissue Mammography With Complementary Breast US Examination Of Both Breasts Revealed:

- ACR c: heterogeneous fibroglandular parenchymal densities.

Left breast:

- A circumscribed hyperdense lesion with partially obscured is seen occupying the inner quadrants. By U/S, it appears as a globular heterogeneously hypoechoic lesion showing cystic cliffs. It is seen subcutaneous with no skin invasion or significant thickening. It measures 10x9.5x7cm.
- Unremarkable nipple-areola complex
- No spiculated breast mass
- No axillary lymphadenopathy.
- No infra- or supraclavicular lymphadenopathy

Right Breast:

- No pathological microcalcification clusters.
- No solid lesions.
- A few anechoic scattered cysts are noted in the UOQ
- Normal central ducts system.
- Normal skin thickness and contour as well as nipple areola complexes.
- Nonspecific left axillary lymph-nodes.

Opinion:

- **Left breast pathologically proven neoplastic mass for correlation to histopathological results (BIRADS 6)**
- **Mild fibrocystic changes of right breast (BIRADS 2)**

Best regards,
Dr. Amira H. Radwan M.D
Breast Imaging Consultant,
NCI, Cairo University

خط الساخن
19182
عمـارة كنتاكى
عمارات المروة - حلوان
كمبوند وسط البلد
www.horusradiologycenter.com